STOCK PITCH · SYSTEMATIC VALUE PORTFOLIO · 29 OCTOBER 2026

Centene

NYSE: CNC · Health Care / Managed care · United States

Cheap for the wrong reason: the screen read stale earnings, and the recovery came from elsewhere

NEUTRAL Case study

Price	**$67.60**
Target (base case)	**$65.00**
Upside / downside	**−4%**
Horizon	**12 months**

Price: intraday, 29 Oct 2026

$33.5bn
Market cap

> $5.20
2026 adj. EPS guide

13.0x
P/E on the guide

−8%
Membership, y/y

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Case study: Cheap for the wrong reason

NEUTRAL

$65.00

base-case value, −4% vs $67.60

In December 2025 Centene was the cheapest health-care stock in our universe, on FY2024 earnings that no longer existed. 2025 adjusted EPS collapsed to $2.08 before a 2026 repricing rebound to a guided floor above $5.20. The stock has recovered; at $67.60 it is fairly priced, and the lesson is about process.

1

Why the model bought it

With a six-month accounting lag, the December 2025 score used FY2024 accounts: pre-collapse EBIT and book equity before a $6.7bn goodwill impairment. EV/EBIT, P/B and E/P all screened at the top of the sector.

2

What actually happened

In July 2025 Centene withdrew guidance on Marketplace risk adjustment and Medicaid cost trends. 2025 adjusted EPS came in at $2.08. Repricing then worked: the 2026 floor was raised four times, from $3.00 to $5.20.

3

Where it stands now

At $67.60 the stock trades at 13x a 2026 figure that includes about $0.50 of non-recurring items. Membership is shrinking (−8% y/y) and Medicaid still runs at a 93.6% benefits ratio. Upside and downside look balanced.

Centene at a glance

Medicaid

Largest US Medicaid managed-care organisation: 11.9m members at September 2026, down from 12.7m.

Marketplace (ACA)

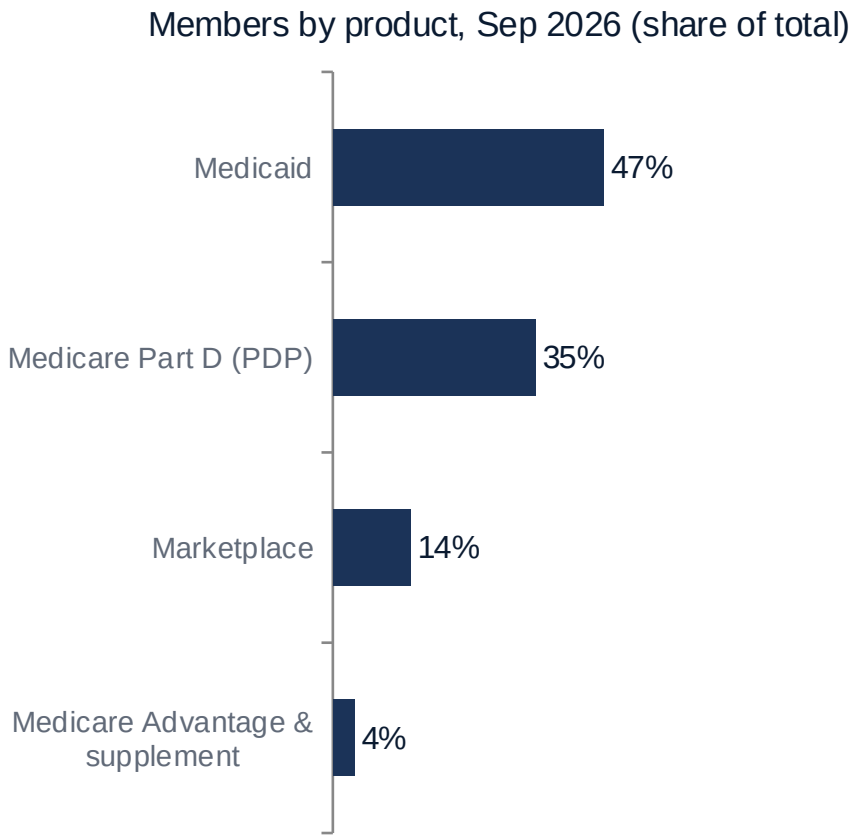
Ambetter plans: 3.4m members versus 5.8m a year earlier after enhanced premium tax credits expired.

Medicare

Wellcare: 0.95m Medicare Advantage and supplement members plus 8.9m stand-alone Part D members.

Other

Remaining Magellan Health businesses held for sale ($513m impairment, December 2025).



Source: company filings; Members by product, Sep 2026

What moves the stock beyond the company

−42%

ACA subsidies

Marketplace membership y/y after enhanced premium tax credits expired on 31 Dec 2025.

2027

Medicaid reform

OBBBA work requirements and more frequent eligibility checks phase in from end-2026.

+2.48%

Medicare rates

CMS final 2027 Medicare Advantage rate notice (April 2026), versus +0.09% proposed.

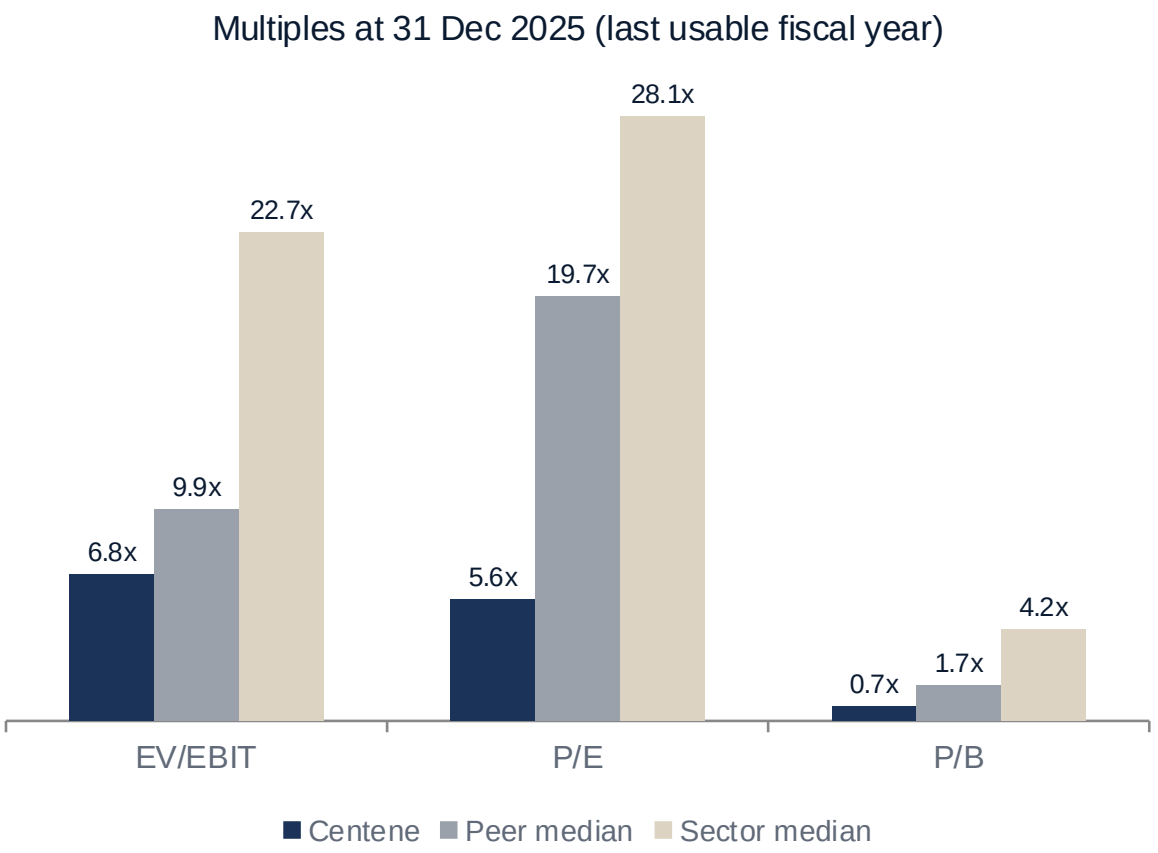
93.6%

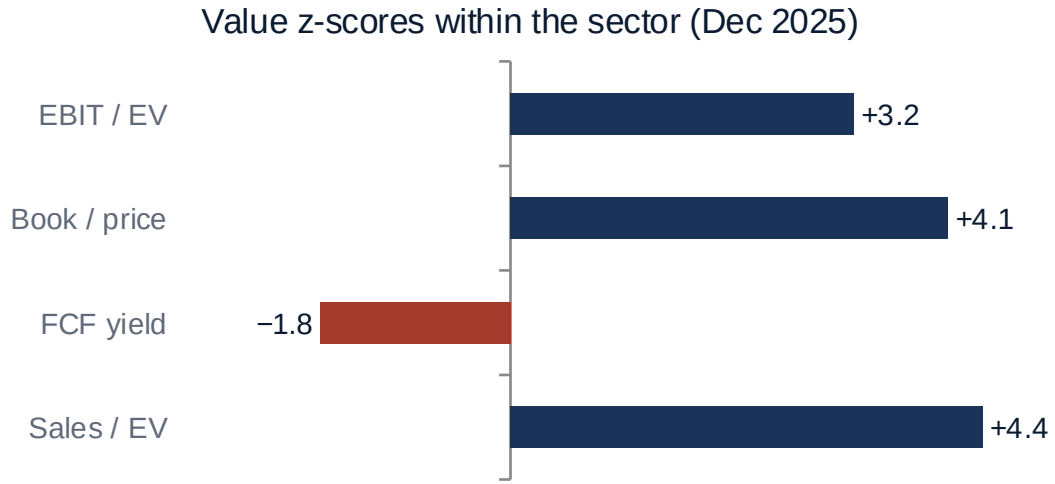
Medical cost trend

Medicaid benefits ratio in Q3 2026: behavioural health, home health and high-cost drugs.

Source: company releases and research brief (see Sources slide)

Ranked 1 of 108 in Health Care on our value score

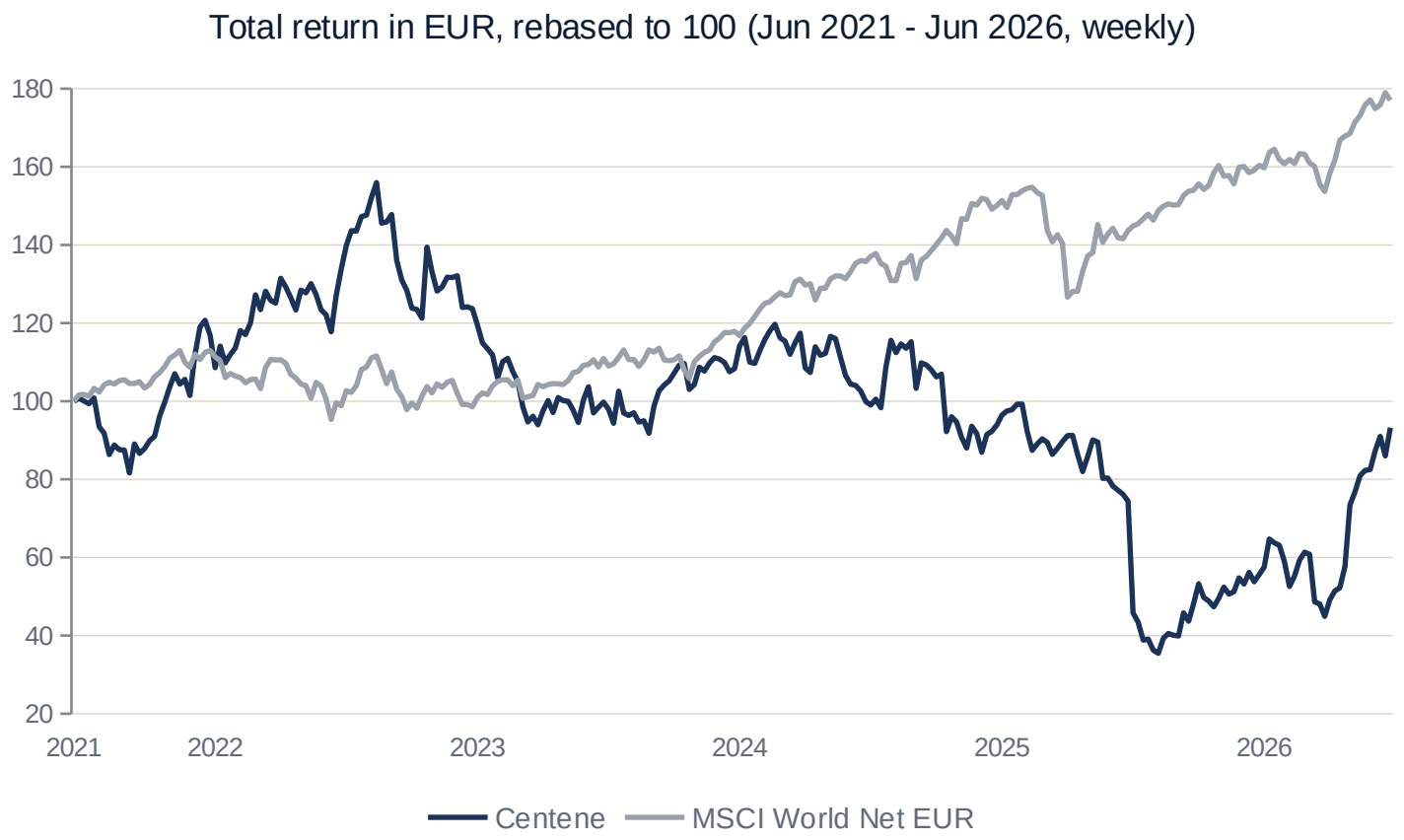




Composite score 2.47, first of 108 in its sector. The score averages the z-scores above (four ratios as yields). Accounts are used only from fiscal year end + 6 months, so in December 2025 the screen read the accounts for the fiscal year ended December 2024.

Source: Portfolio Project pipeline (Bloomberg data); peers: Elevance Health, Humana, Cigna, UnitedHealth, CVS Health (pipeline peer set).

Held at 10 of 10 rebalances since June 2021



+62.0%

H1 2026 return (EUR)

+10.8%

MSCI World Net EUR, H1 2026

2.0%

Weight, equal weight

1.7% / 0.5%

Min-variance / mean-variance

Source: pipeline weekly total returns (EUR, net of withholding tax) and portfolio weights at the 31 Dec 2025 rebalance

Reported numbers and guidance

	FY2025	9M 2026	Q3 2026	2026 guide
Total revenues	$194.8bn	$154.2bn	$50.6bn	$196.5-200.5bn
Health benefits ratio	91.9%	89.9%	92.7%	90.4-91.1%
Adjusted diluted EPS	$2.08	$6.45	$0.58	> $5.20
GAAP diluted EPS	−$13.53*	$5.15	−$0.14	> $3.46
Total debt	$17.4bn	—	$15.5bn	—
Debt / capital	46.5%	—	41.1%	—

*Includes a $6.7bn non-cash goodwill impairment (Q3 2025). No dividend. Sources: Centene FY2025 and Q3 2026 releases.

What the bulls say, and what the evidence says

THE BULL ARGUMENT	WHAT THE EVIDENCE SAYS
“The stock has doubled, so the model was right”	The 2026 rebound came from repricing and policy, not from the cheapness the model measured. The signal had no defence against a fast fundamental break.
“Managed care is a stable compounder”	Margins are thin: each 10 bp of benefits ratio on ~$175bn of premiums is about $175m of pre-tax profit.
“Guidance raises mean upside”	The floor format ('greater than $5.20') and ~$0.50 of non-recurring items flatter 2026; FY2027 guidance in February is the real test.

Base-case value $65.00 per share, −4% vs today

Method: P/E on 2027 adjusted EPS

Scenario	Key inputs	Value / share	vs price
Bear	EPS $3.50 × 11x Medicaid margins stay under water; risk-adjustment shock	**$38.50**	**−43%**
Base	EPS $5.00 × 13x 2026 earnings power holds ex one-offs	**$65.00**	**−4%**
Bull	EPS $6.50 × 14x Medicaid rates catch up; buybacks resume	**$91.00**	**+35%**

Managed-care insurers are valued on earnings. We apply peer-range P/E multiples to three 2027 adjusted EPS outcomes: a reset below the 2026 floor ex one-offs (~$4.70), a flat year, and continued margin repair.



Source: own valuation; 52-week range and consensus from third-party data (29 Oct 2026)

09 CATALYSTS

What could move the stock in the next 12-18 months

Nov 2026-Jan 2027

ACA open enrolment for 2027 (median premiums +15%)

Feb 2027

Q4 2026 results and first FY2027 guidance

From Jan 2027

Medicaid work requirements and eligibility checks

Apr 2027

Final 2028 Medicare Advantage rate notice

Jun-Jul 2027

Risk-adjustment data for benefit year 2026

The number to watch: FY2027 guidance materially below ~$4.70 of adjusted EPS

What would change our view

Risk	Mitigant
Marketplace risk pool worsens as healthier members lapse	Repricing already filed; membership smaller
Medicaid rate adequacy versus acuity	State rate increases in 2026
Low Medicare Star ratings limit bonuses	Small MA book (0.95m members)
Little unregulated cash ($175m)	Debt reduced from $17.4bn to $15.5bn

WE WOULD CHANGE OUR VIEW IF

- FY2027 guidance materially below ~$4.70 of adjusted EPS
- Medicaid benefits ratio failing to improve from ~93.5%
- Another mid-year risk-adjustment shock

Sources

[1] Centene, "Centene Corporation Reports Third Quarter 2026 Results", 8-K Ex. 99.1, 27 Oct 2026

[2] Centene, "Centene Corporation Reports 2025 Results and Announces 2026 Guidance", 8-K Ex. 99.1, 6 Feb 2026

[3] Centene, Form 10-Q for the quarter ended 30 Sep 2026 (financial statements and notes only), filed Oct 2026

[4] Centene, "Centene Corporation Withdraws 2025 Guidance", 8-K Ex. 99.1, 1 Jul 2025

[5] Centene, "Centene Corporation Reports Third Quarter 2025 Results", 8-K Ex. 99.1, 29 Oct 2025 (content via search excerpt)

[6] Centene, "Centene Corporation Reports First Quarter 2026 Results", 8-K Ex. 99.1, 28 Apr 2026 (content via search excerpt)

[7] Centene, "Centene Corporation Reports Second Quarter 2026 Results", 8-K Ex. 99.1, 28 Jul 2026

[8] Fierce Healthcare, "CMS gives Medicare Advantage rates a 2.48% bump for 2027 plan year in final rule", Apr 2026

[9] American Hospital Association, "CMS releases 2027 Medicare Advantage, Part D star ratings", 9 Oct 2026; Becker's Payer Issues, 2027 star ratings coverage, Oct 2026

[10] Fierce Healthcare, "Centene posts $71M loss, $50.6B in revenue for Q3", Oct 2026 (page blocked; content via search excerpt)

[11] Becker's Payer Issues, ACA enhanced subsidy coverage (expiry 31 Dec 2025; talks collapsed per WSJ, 5 Feb 2026); Fierce Healthcare, "Centene, Oscar Health outline 2027 ACA marketplace plans" (KFF 15% median 2027 increase), 2026

[12] stockanalysis.com, Centene (CNC) quote page, data as of 29 Oct 2026 (third-party)

Portfolio Project pipeline: scores, multiples, weights and returns computed from Bloomberg data extracted on the SKEMA terminal (Oct 2026).

Disclosure: student research prepared for educational purposes and job interviews. It is not investment advice or a solicitation. Third-party market data (prices, consensus, forward multiples) is labelled and was accessed on 29 October 2026; third-party figures should be checked against company filings or a Bloomberg terminal before use. Backtest results are hypothetical.